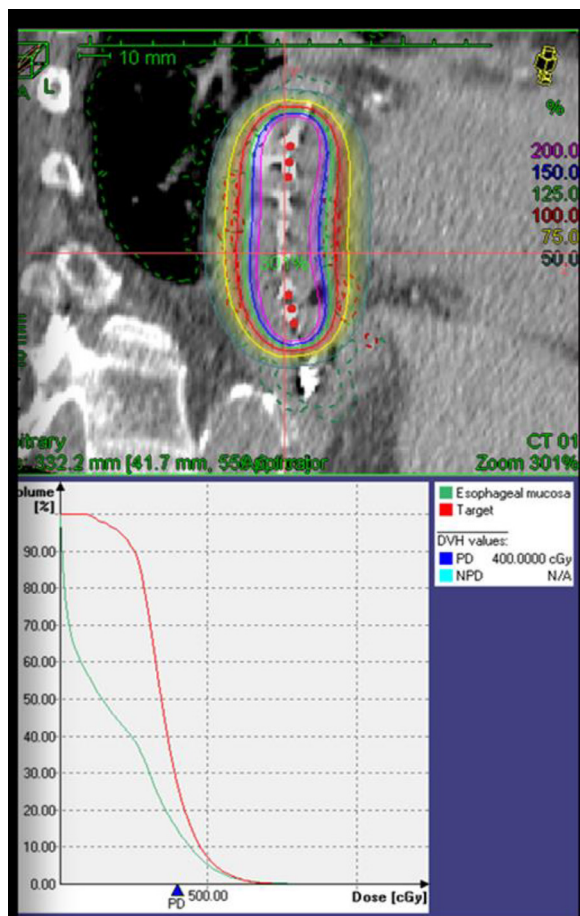


Results: 20 patients were included in this study. Median follow up was 6 months (range 4–36 months). 4/21 (25%) were females and 16/20 (75%) males. 76 fractions were treated in total. 3/20 (15%) patients received 5 fractions, 4/20 (25%) received 4 fractions, 9/20 (45%) received 3 fractions and 4/20 (25%) received 2 fractions of 5 Gy each. 7/76 (10%) fractions with a conventional 8mm single channel catheter and 69/76 (90%) were treated with balloon catheter. 4/20 (25%) patients were alive at the time of this report. 80% of patients showed improvement in dysphagia. Stenosis requiring dilatation occurred in 60%. Stenosis requiring stent occurred in 50% of either due to local progression of disease or stricture formation at least temporarily. 2/20 (10%) patients were found to have fistula formation. First patient that developed a tracheoesophageal fistula at 12 months, had a history of tracheal involvement at the initial diagnosis. Second patient that developed a mediastinal fistula at 4 months was treated for postoperative anastomotic recurrence and all his fractions were treated with narrow lumen conventional catheter

Conclusions: In select patients, especially medically inoperable with early stage primary cancers or patients with recurrent or persistent disease, endoluminal HDR brachytherapy can be well-tolerated treatment option with low rates of toxicity. Widespread use of esophageal HDR brachytherapy has decreased use due to toxicity, but the use of newer balloon catheters and CT planning might contribute to our lower toxicity. Prospective study is warranted to determine the role of this technique in long-term control and cure of esophageal cancer as systemic and targeted therapy options offer better control of systemic disease and local control remains challenging.



MSOR7 Presentation Time: 5:15 PM

Ocular Brachytherapy with I-125 an Increasingly Important Option in Ocular Melanoma: A Case Series from a Low Middle Income Country, Latin American Experience

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Purpose: To evaluate the oncological results in local control and vision preservation at 5 years with ocular brachytherapy treatment with I-125 in the largest cohort of patients in Colombia and one of the largest in Latin America, given that the National Institute of Cancerology Bogotá-Colombia has an important recognition in being the only institution that performs this management in the country. In an emerging country with limited resources, where surgical management with exenteration for some neoplasms such as uveal melanomas is the treatment standard, ocular brachytherapy is becoming increasingly important.

Materials and Methods: This was a retrospective review of all patients with ocular melanomas treated with intraocular low-dose-rate (HDR) I-125 brachytherapy from July 2013 to October 2022 at the National Institutes of Medicine, cancerology, Bogotá-Colombia. 51 patients with ocular melanoma were identified. The median age at diagnosis was 53 years (range=21–62). The majority were choroidal melanomas (77%, n=39), ciliary body melanomas (19%, n=10), and two iris melanomas (4%, n=2). Average hospitalization during the procedure was 3 days. The most widely used apex radiation dose prescribed was 85Gy ($\pm$: 10.3 Gy) using the COMS plate. All patients had individualized planning based on CT, fused with fine-slice resonance, ultrasound.

Results: The mean follow-up for each patient varied from the first treated patients at 71.5 months ($\pm$: 51.8 months). 47 patients (92.1%) remained free of recurrence. 41 (80.3%) demonstrated complete tumor regression after a median period of 24 months (3–74 months). At the final examination, the visual acuity of 33 patients (64.7%) was 20/20 and 14 patients (27.4%) had a visual acuity of 20/40. Retinopathy was detected in 4 patients (7.8%). After the treatments, only two patients (3.9%) had active vascular changes at the last examination. Moderate optic neuropathy was observed in 1 patient (1.9%). Cataract development was diagnosed in 10 patients (19.6%) and 5 patients (9.8%) had bilateral cataract development.

Conclusions: Ocular brachytherapy with I-125 for ocular melanoma is an excellent treatment strategy for properly selected patients, offers optimal local control, preserves the visual, functional and aesthetic capacity of patients, our data on oncological results support this option every time more important in developing countries such as Colombia where access to this management modality is limited.

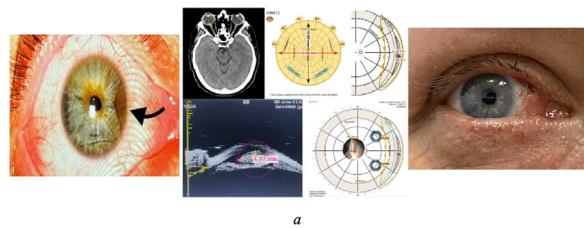


Figure 1. Patient with ocular iris melanoma, planning with computerized CT, ultrasound evaluating depth, planning and results with 85% regression of the lesion in 6 months. (a).